

RETINAL EDGE TRIAGE

Explainable AI-Assisted Diabetic Retinopathy Screening for Rural Healthcare
Official Clinical Triage Report | Generated: 04-Sep-2026 14:33 UTC

Patient ID: RET-2026-000020	Full Name: Test Screening Patient	Age / Gender: 52 / Female
Screening ID: SCR-2026-000019	Location: Test Rural PHC	Diabetes Type / Duration: Type 2 (6 years)

1. Retinal Image Acquisition & Quality Gate

Eye Evaluated: Right Eye	Image Source: Portable Fundus Camera	Quality Score: 67.4%	Quality Status: BORDERLINE
Focus / Blur: 31.1%	Illumination: 52.4%	Vessel Visibility: 80.1%	Field of View: 91.5%

2. AI Screening Assessment (APTOS 2019 & IDRiD Models)

DR Severity Grade:	Grade 1 — Mild DR	Referable DR:	NO (Routine Follow-up)
AI Confidence:	89.0%	DME Risk Status:	Possible DME detected (Multiple hard exudates identified within parafoveal region, indicating potential macular edema risk.)
Detected Lesions (IDRiD):	Microaneurysms: 9 Hemorrhages: 3 Hard Exudates: 16 Soft Exudates: 1	Model Version:	Demo DR Classifier (APTOS 2019 Arch) v1.0

3. Human-in-the-Loop Ophthalmologist Adjudication

Doctor Decision:	ACCEPTED	Reviewing Doctor:	Dr. Ophthalmologist
Final Grade Assigned:	Grade 1	Override Justification:	None
Clinical Notes:	AI analysis matches clinical presentation.	HCW Instructions:	None

4. Risk Triage & Clinical Referral Plan

Risk Category:	LOW RISK	Referral Priority:	URGENT
Referral ID:	REF-2026-00011	Recommended Facility:	District Eye Hospital
Triage Reason:	Mild non-proliferative changes. Strict glycemic control and 6-month re-screening.	Follow-up Schedule:	6 Months

STATUTORY CLINICAL NOTICE: This report is generated by an artificial intelligence screening triage prototype (Retinal Edge Triage). AI predictions DO NOT constitute a definitive medical diagnosis. Final clinical interpretation and management must always be performed by a qualified ophthalmologist. Compliant with rural screening clinical governance principles.